



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : سوزان عبدالعظيم محمود حمزه

المحافظة : القاهرة الحي / المدينة : مصر الجديده العنوان : الحجاز

رقم البطاقة : 26604260101448 رقم التليفون : 01022276154 تاريخ تسجيل الإستمارة : 24-09-2026

العمر (تاريخ الميلاد) : 26-04-1966 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي

الاورام المنتشر [14456]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سوزان عبدالعظيم محمود حمزه National ID: 26604260101448	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٢/٠٦

٣٦٦٠٤٢٦٠١٠١٤٤٨

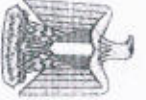
ربة منزل

مقروحة

مستفيدة

مدينت عبدالصمد احمد طاهر

البطاقة سارية حتى ٢٠٢٩/٠٦/٢٩



جمهورية مصر العربية
وزارة الداخلية
بطاقة هوية شخصية

سوزان

عبدالعليم محمود حمزة

٢٥ أوتيج على عبدالرازق ممدان الحجاز

مصر الجديدة - القاهرة

٢٦٦٠٤٢٦٠١٠١٤٤٨

JL3600920



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للصحة
مصر

02/21/2026

MDT FORM

Name:	Souzan Abdelazem Mohamoud Hamza	MRN:	5626
Age:	60 Y	ID:	1336-2024
Menopausal Sta	POSTMENOPAUSAL	Phone:	01022276154
NRS SCORE:	2	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

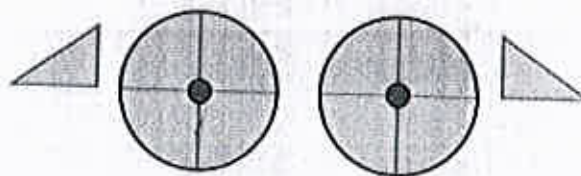
BX: IDC LEFT BREAST
ER: 8/8 **PR:** 7/8 **HER2:** -VE **KI67:**

R

L

Sonomammography:**Mets Workup:****History:**

POSTMLEFT BILAT BREAST CANCER MET TO BONE .HISTORY LEFT BREAST
 CANCER CT4D N2 M1 BONE METS DIAGNOSED ST AJ FROM
 9/2020TILL4/2024 DP ST FASOLDEX IBRANCE FROM 6/2024 TILL 8/2024 DP
 LOCAL PG AND NEWLY DEVELOPPED RT BREAST INDURATION BIOPSIED ILC
 ER8/8 PR8/8 HER2-VE ST TAXOL ,CARBOPLATIN BUT BEC OF GRADE 3 PN
 STOPPED TAXOL 3 WEEKS AGO AND KEPT ON CARBOPLATIN REC UP TO 20
 WEEKSCARBOP AND 16 WKS TAXOL BUT PT HAS LOCAL PD INCREASE
 REDNESS INDURATION OF LEFT BREAST AND BEWLY DEVELOPPED SKIN
 NODULES



TNM

Recommendations:**MDT Outcome & Plan**

for affinitor aromasin

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ هاني عزت					

Patient Name : الأستاذ / موزان عبد العظيم محمود حمزة

Patient No. : 26604260101448

Path No. : 2388/25

Sex : Female

Age : 59 Y

Referring Doc :

Request No. : 13008281

Request Date : 28/07/2025

Verification Date : 02/08/2025

Financial Category : جرساف روسي - نفقة الدولة

Department : Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Right breast mass(UOQ) BIRADS 5

Nature of specimen

Tru cut biopsy, collected at 27/7/2025, 10:30 am

Gross examination

Multiple(4) grayish white tissue cores up to 1 cm, totally submitted.

HC technique: Sections prepared from paraffin block for immune-staining using Bench

mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

Histological type: Invasive lobular carcinoma

Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2

DCIS: No

Angioinvasion: Couldn't be detected in the received biopsy

Perineural invasion: No

IPT (Positive control showed appropriate reactivity) :

ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)

PR: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)

HER2: No membrane reaction in of tumor cells (score 0)

Ki 67: 15 %

Diagnosis

Right breast mass, Tru cut biopsy:

Verified by :

Dr. Heba Hasan

Printing Date : 07/08/2025 12:07:16

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

Pathology Report
[DOCTYPE:PATHOLOGY_REPORT]

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Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Invasive lobular carcinoma, Grade 2
IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 8/8).
- * HER2: Negative (score 0).
- * Ki67: 15 %

Signature

Dr. Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

Printing Date : 07/08/2025 12:07:16

Page 2 of 2

Printing By : Mrs.Mayada Hegazy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



**Patient Name: Suzan Abdel azeem
Mahmoud Hamza**

Patient ID: 101689662

Ref. Phy.:

Study Date: 15/07/2025

- Complementary US revealed skin thickness measuring 6.6 mm with subcutaneous edema noted, it shows an irregular hypoechoic mass lesion noted deep seated at 1 o'clock position, measuring 3.3x2.3 cm, likely malignant lesion. It shows enlarged suspicious axillary LN measuring 2x1.2 cm with cortical thickness measuring 6mm.

Conclusion:

Right breast suspicious UOQ NMLE reaching the nipple, for histopathological correlation BIRADS 4.

left breast suspicious microcalcifications & NMLE as described BIRADS 6.

Bilateral suspicious axillary LNs.

MRI breast with contrast is recommended.

Much Obligated,

Asmaa Magdy, MD



Patient Name: Suzan Abdel azeem
Mahmoud Hamza
Ref. Phy.:

Patient ID: 101689662
Study Date: 15/07/2025

**BILATERAL BREAST CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY (CESM) WITH COMPLEMENTARY
LEFT HIGH RESOLUTION BREAST ULTRASOUND**

Clinical History: 59-year-old female, known left breast cancer not operated, metastatic to bone, on hormonal treatment. no previous studies for comparison.

Procedure:

Low energy and high energy followed by algorithmic recombined images were taken after IV injection of low osmolar iodinated contrast, 1.5 ml/kg body weight.

Findings:

ACR density C heterogeneous glandular density breast parenchyma.

Background parenchymal enhancement (BPE): moderate symmetrical enhancement.

Right breast

Low energy images (LE) without contrast

- Marked diffuse skin thickness & retracted nipple.
- No speculated densities or suspicious microcalcifications.

Recombined images (RI) with contrast:

- The breast shows UOQ segmental non mass enhancement reaching the nipple.
- No suspicious mass enhancement. US interrogation revealed right breast skin thickening measuring 5.6 mm & retro areolar duct ectasia measuring 5.7 mm & UOQ altered echogenicity with distortion noted at 11 o'clock position, 5cm from the nipple, measuring 10x15 mm. Right axillary suspicious enlarged LN measuring 2.1x1.1 cm with thickened cortex measuring 5mm.

Left breast

Low energy images (LE) without contrast

- Marked diffuse skin thickness and retracted nipple with suspicious grouped microcalcifications noted at central region 5 cm from the nipple, measuring 3.5x3.3 cm.

Recombined images (RI) with contrast:

- Non-mass pathological contrast uptake is noted at area of grouped microcalcifications at central region & another UOQ deep seated patchy enhancement is noted.

Name: Mrs/ Suzan Abd El Azeem Hamza . **Date:** 16/7/2025
Ref. By: Prof. Dr. Tamer El Nahas.

MULTI-SLICE FDG PET/CT Examination

Clinical history:-

- A 56-year-old female, gave history of treated left breast carcinoma, received target & hormonal therapy.

Reason of study:

- FU PET-CT evaluation.

Technique:-

- ❖ Approximately one hour after IV administration of around 8 mCi of f-18 FDG. Whole body PET was acquired from the skull level till the mid-thighs. Both arms were placed in extension position above the heads. CT was also done aiming at anatomic localization and attenuation correction and obtaining fusion images. Data was processed and displayed into the usual cuts.

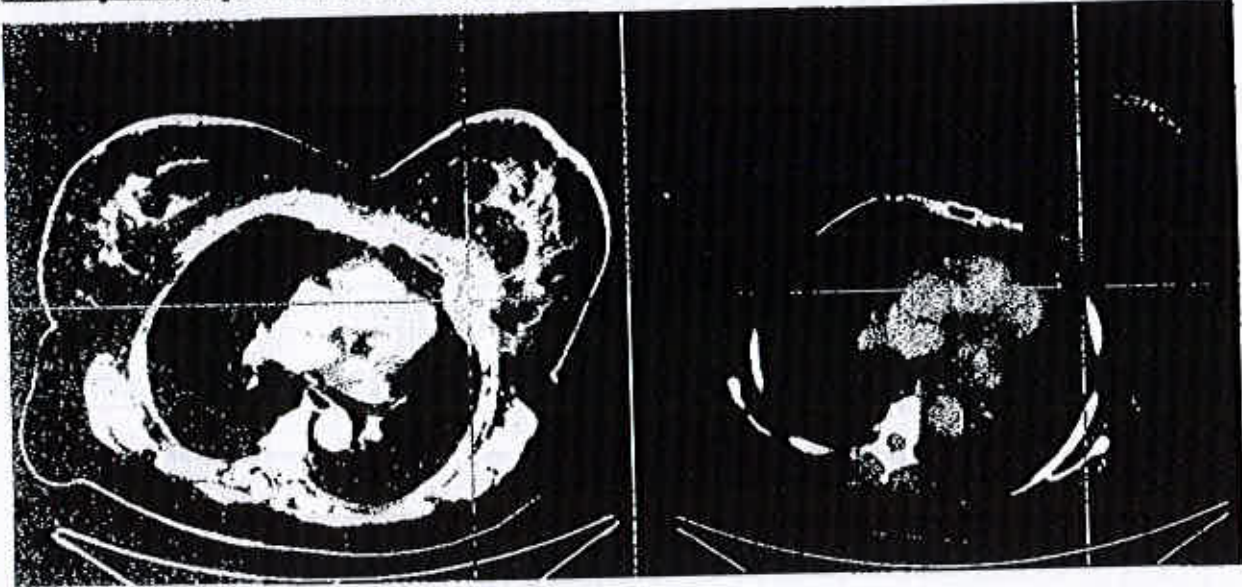
Weight:	95 Kg	Height:	170Cm
Bl.Gluc.level	220	IV Contrast	No

(+2)0155 1515 732 - (+2)0100 7844 701 - (+2)0122 8290 029 - (+2)24140045 - (+2)24140046

المستشفى المركزي - شارع عمر ابن الخطاب - ارض الجولف - بجوار سبلي ستارز - مدينة مصر

**❖ Findings:-**

As compared to previous PET-CT study dated 18/4/2024, current study reveals:

**I-Both breasts /1st site of concern:**

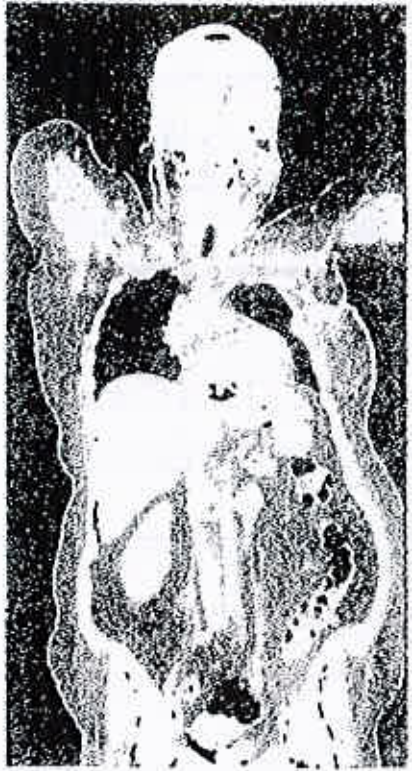
- Left breast shows low active (FDG avid) confluent soft tissue lesion resending on pectoral muscle, measuring about 3.2 x 2.1 cm with max SUV~4.2. (as compared to 3.3 x 2 cm with max SUV~4 at previous study) .
- Associated pectoral intra-thoracic extension with pleural nodule measuring about 4 x 1.6 cm with max SUV~3.4
- The contra lateral right breast showing no metabolically active (FDG avid) lesions.
- Metabolic remission regarding previously seen bilateral axillary lymph nodes, for FU.

II-Head :

- Uniform tracer bio-distribution over the scanned sections of the cerebral and cerebellar hemispheres.
- No metabolically active (FDG avid) masses are noted.

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المستشفى المركزي - شارع عمر ابن الخطاب - ارض الجولف - بجوار سبلي ستارز - مدينة نصر



III-Neck: -

- No metabolically active (FDG avid) cervical lymph-nodes are noted.
- Normal physiological tracer distribution over the scanned pharyngeal lymphoid tissue as well as salivary glands.

IV-Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses.
- No metabolically active (FDG avid) hilar or mediastinal lymph-nodes.
- No metabolically active (FDG avid) pleural or pericardial effusion.
- Normal appearance of chest wall with no related FDG avid lesions.

V-Abdomen and pelvis:

- Enlarged liver showing no metabolically active (FDG avid) focal hepatic lesions.
- Average size spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active (FDG avid) abdominal or pelvic lymph nodes.

(+2)0155 1515 732 - (+2)0100 7844 701 - (+2)0122 8290 029 - (+2)24140045 - (+2)24140046

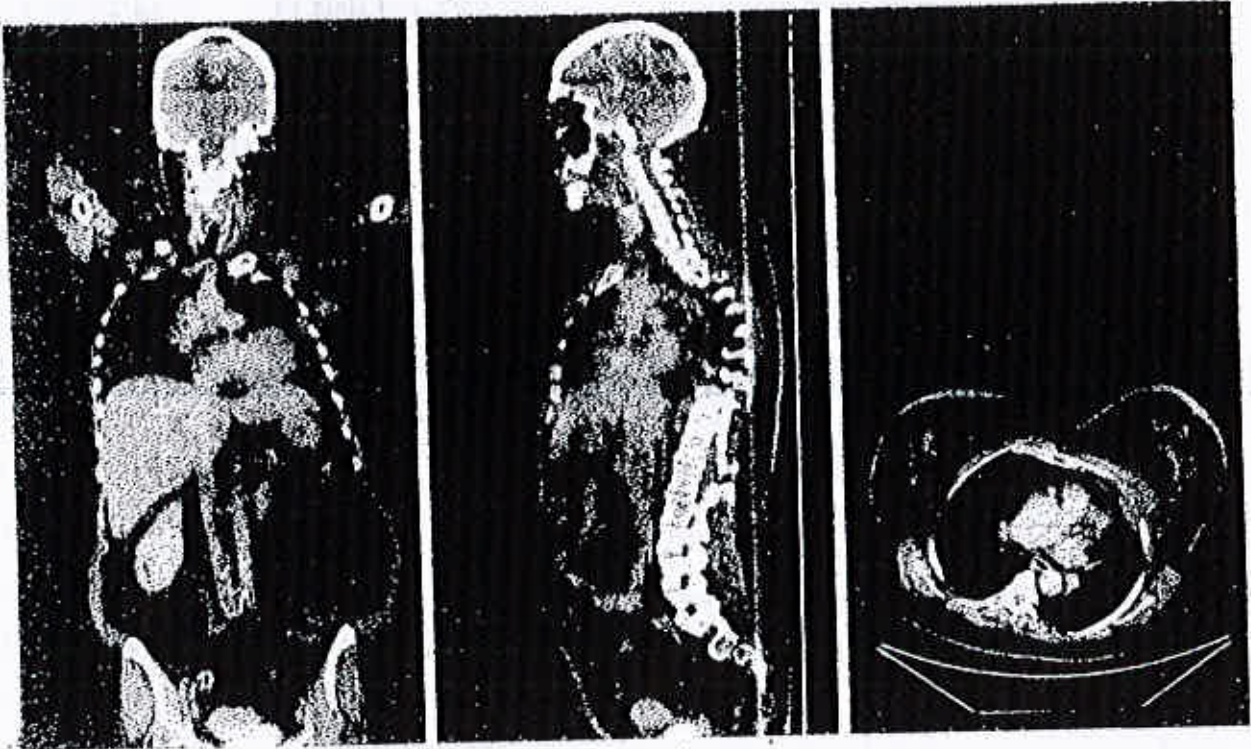
المستشفى المركزي - شارع عمر ابن الخطاب - ارض الجولف - بجوار سيلي ستارز - مدينة نصر



- Normal tracer concentration and excretion at both kidneys, ureters and urinary bladder.
- Normal appearance of the pancreas, kidneys, adrenals, aorta and IVC.
- Calcular gall bladder.
- Bulky uterus, for further sonographic assessment.
- No metabolically active (FDG avid) abdominal ascites.

VI-Scanned axial and Appendicular skeleton:-

- No metabolically active (FDG avid) lytic or sclerotic lesions.
- Old rib fractures are seen at right 3rd & left 7th ribs.



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المستشفى المركزي - شارع عمر ابن الخطاب - ارض الجولف - بجوار سيلي ستارز - مدينة نصر

**∴ Conclusion: -**

∞ Follow up PET-CT study of a known patient with left breast carcinoma showing:

- ✓ Left breast shows low active (FDG avid) soft tissue lesion resending on pectoral muscle , with underlying focal right pleural thickening as described (? tumoral recurrence. For further histopathological correlation) .
- ✓ Metabolic remission of previously seen bilateral axillary lymph nodes.
- ✓ No other metabolically active (FDG avid) metastatic deposits.

PET-CT Unit

Dr. Khaled Yousef.
Radiologist.

Dr. Hany Maurice .
Nuclear Medicine Consultant
University of London.

(+2)0155 1515 732 - (+2)0100 7844 701 - (+2)0122 8290 029 - (+2)24140045 - (+2)24140046

المستشفى المركزى - شارع عمر ابن الخطاب - ارض الجولف - بجوار سبلى ستارز - مدينة نصر

Name: Mrs/ Suzan Abd El Azeem Mahmoud. Date: 3/9/2026.
Ref.Prof.Dr.: Tamer El Nahas .

MULTI-SLICE FDG PET/CT Examination

Clinical history:-

- A 60-year-old female, gave history of both breast carcinoma, received CTH , target & hormonal therapy.

Reason of study:

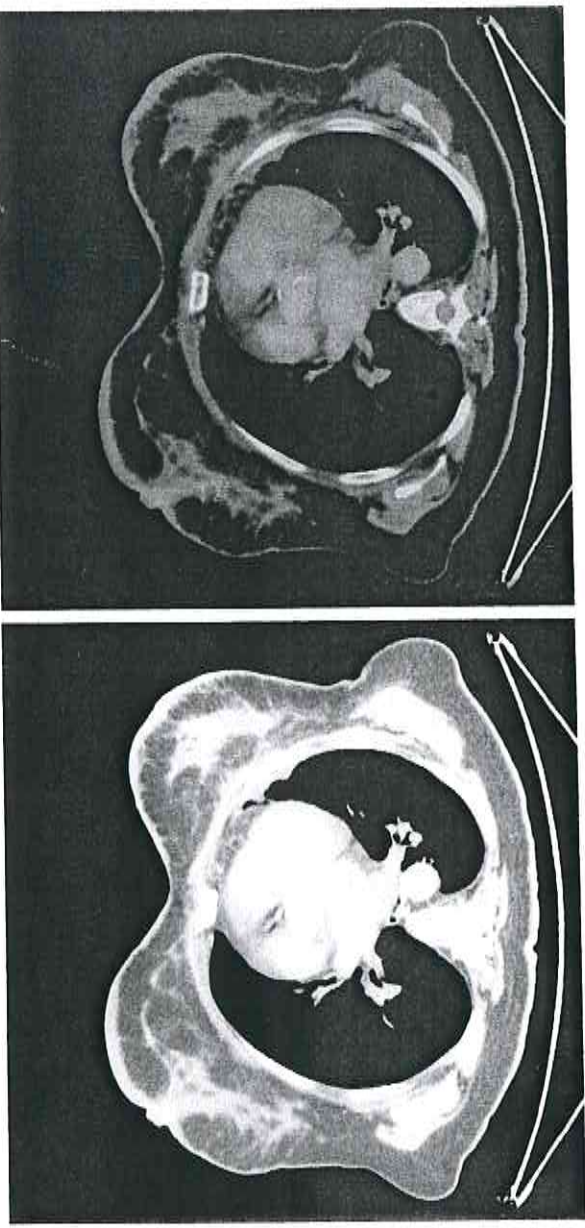
- Follow up PET-CT .

Technique:-

- ❖ Approximately one hour after IV administration of around 5 mCi of f-18 FDG. Whole body PET was acquired from the skull level till the mid-thighs. Both arms were placed in extension position above the heads. CT was also done aiming at anatomic localization and attenuation correction and obtaining fusion images. Data was processed and displayed into the usual cuts.

❖ Findings:-

As compared to previous PET-CT study dated at 18/6/2026 , current study reveals:



I-Both breasts /1st site of concern:

- Left breast shows diffuse edematous parenchymal tissue with overlying mild thickened skin of low grade activity , however no metabolically active (FDG avid) loco-regional tumoural residue/recurrence.
- The contra lateral right breast also showing inactive retro-areolar parenchymal tissue thickening with overlying mild thickened skin .
- No active (FDG avid) bilateral axillary lymph nodes are noted.

II-Head :

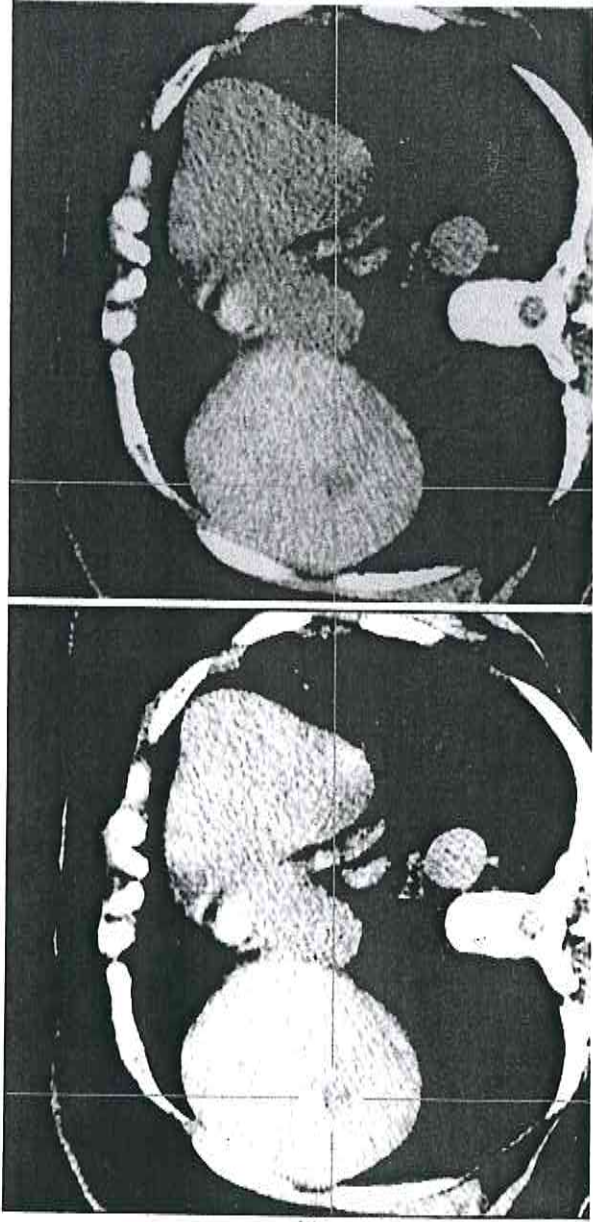
- Uniform tracer bio-distribution over the scanned sections of the cerebral and cerebellar hemispheres.
- No metabolically active (FDG avid) masses are noted.

III-Neck: -

- No metabolically active (FDG avid) cervical lymph-nodes are noted.
- Normal physiological tracer distribution over the scanned pharyngeal lymphoid tissue as well as salivary glands.

IV-Chest:-

- No metabolically active (FDG avid) pulmonary nodules or masses.
- Focal lung lingual mild pleural thickening with underlying atelectatic bands , likely post-irradiation fibrotic sequel.
- No metabolically active (FDG avid) hilar or mediastinal lymph-nodes.
- No metabolically active (FDG avid) pleural or pericardial effusion.
- Normal appearance of chest wall with no related FDG avid lesions.

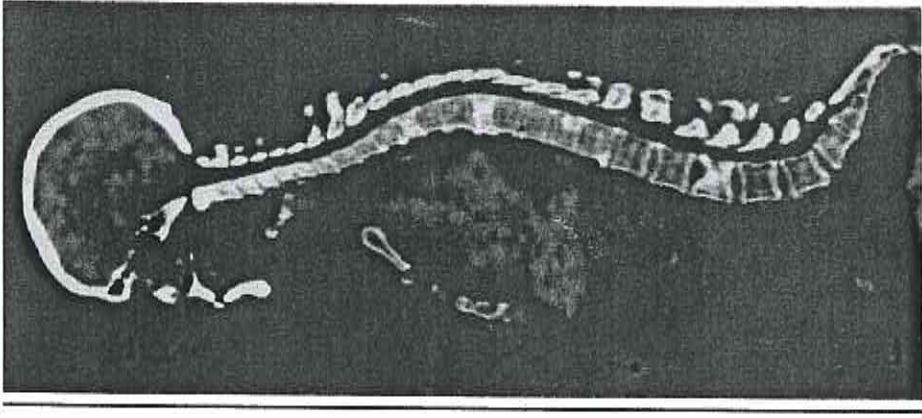
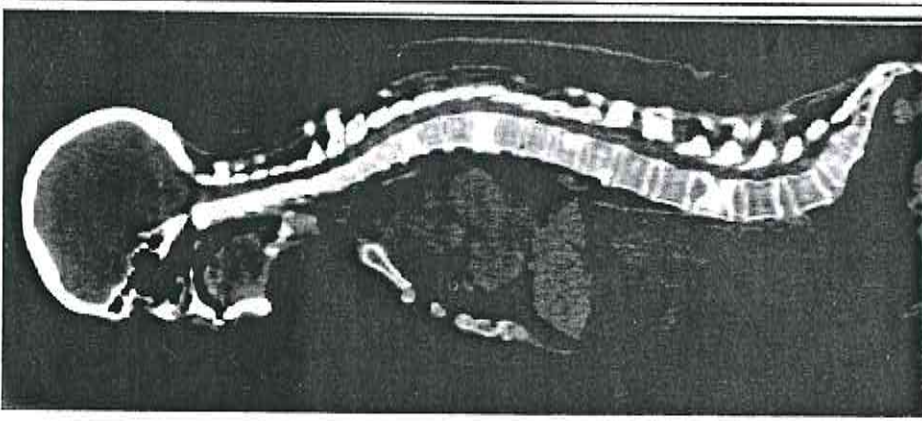
**V-Abdomen and pelvis:**

- Enlarged coarse liver showing right lobe inactive hypodense focal lesions at segments (VIII , VII & V) , averaging about 1.7 cm with no corresponding significant FDG activity , ?remitted ? benign .
- Calcular gall bladder.
- Average size spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active (FDG avid) abdominal or pelvic lymph nodes.

- Normal tracer concentration and excretion at both kidneys, ureters and urinary bladder.
- Normal appearance of the pancreas, kidneys, adrenals, aorta and IVC.
- Bulky uterus, for further sonographic assessment.
- No metabolically active (FDG avid) abdominal ascites.

VI-Scanned axial and Appendicular skeleton:-

- No metabolically active (FDG avid) lytic or sclerotic lesions.
- Still seen remitted inactive sclerotic osseous lesions involving multi-level dorsal & lumbar vertebrae with vertebral wedging at DV8 & LV3 .



∴ Conclusion: -

∞ Follow up PET-CT study of a known patient with left breast carcinoma showing:

- ✓ Both breasts show no metabolically active (FDG avid) tumoural residue /recurrence.
- ✓ Few inactive hepatic focal lesions with no corresponding significant FDG activity as described , ?remitted , ? benign , for further dynamic MRI correlation .
- ✓ No metabolically active (FDG avid) metastatic deposits.

Much Obligated;**PET-CT Unit**

Dr. Khaled Yousef.
Radiologist.

Dr. Hany Maurice .
Nuclear Medicine Consultant
University of London.

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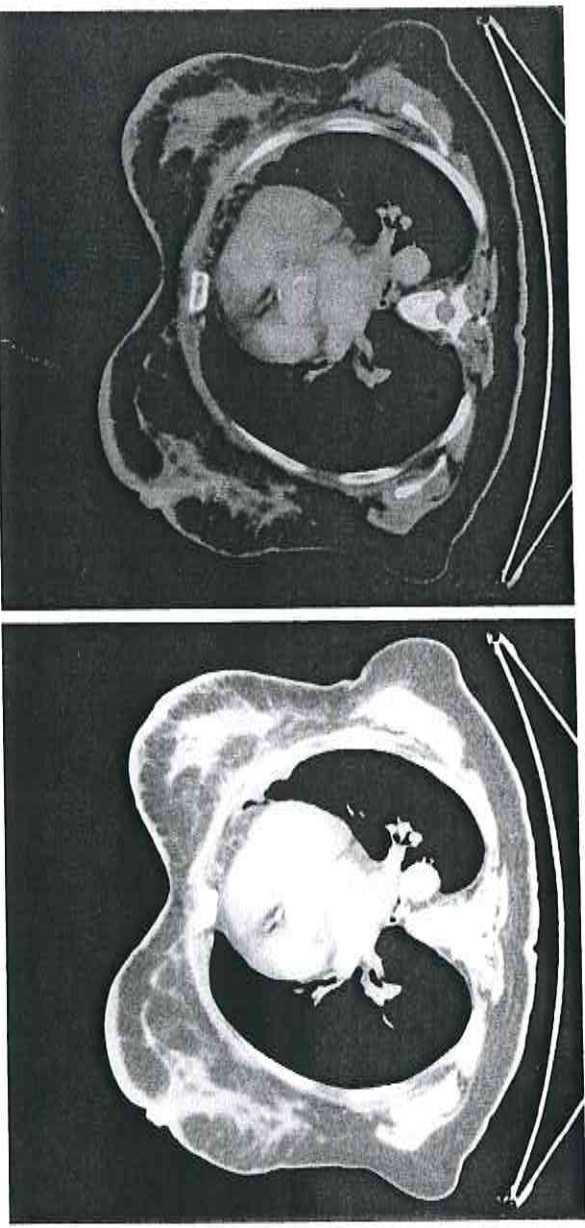
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- No active (FDG avid) bilateral axillary lymph nodes are noted.

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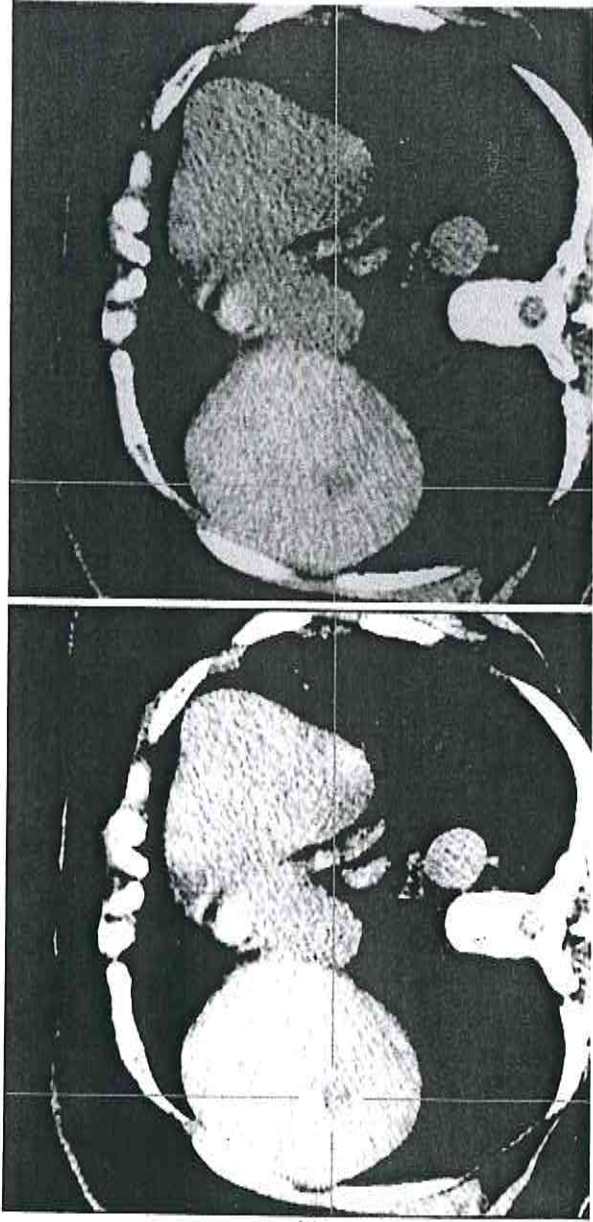
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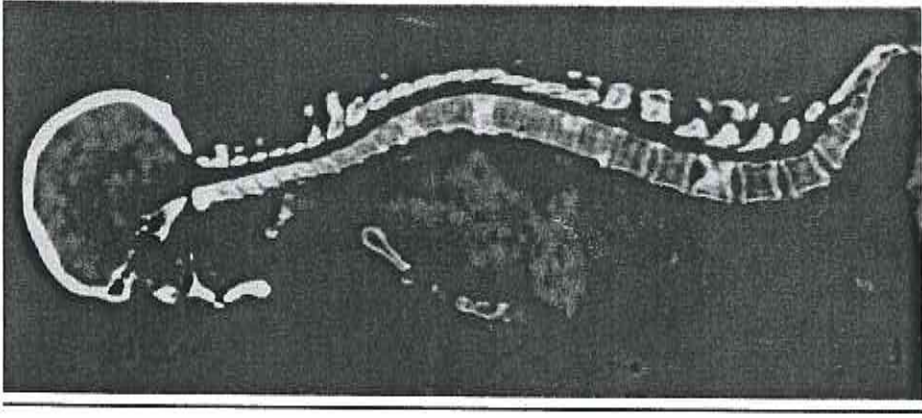
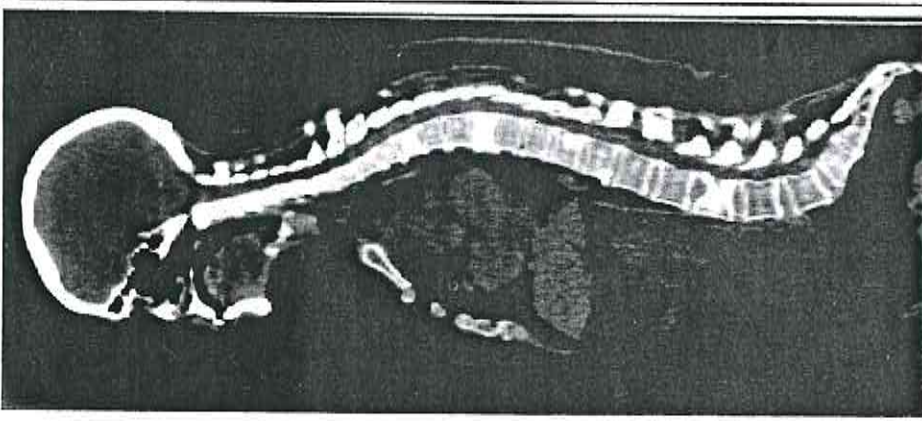
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